



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : سلوي احمد مصطفى حسين

العنوان : ~

الحي / المدينة : قليوب

المحافظة : القليوبية

تاريخ تسجيل الإستمارة : 01-10-2026

رقم البطاقة : 27804051402468 رقم التليفون : 01124254437

العمر (تاريخ الميلاد) : 05-04-1978 المهنة : بدون عمل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي
التوقيع: (.....)

المقرر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان

الوظيفة :
الوظيفة :
الوظيفة :

التوقيع :
التوقيع :
التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :

قسم :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: سلوي احمد مصطفى حسين National ID: 27804051402468	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

٢٠٢٢/١٢
٢٧٨٠٤٠٥١٤٠٢٢٦٨
حامله على نيلوم صانع
مستلمة
إنتى محمد اسماعيل على شعور
٢٠٢٢/١٢/١٢
الطائفة سارية حتى
١١




DMS

← لومين فصارا + لومين
← مقلبي

جمهورية مصر العربية
بكالوريوس الحقوق
سولى
احمد مصطفى حسن حرب
قلم
مركز حقوق - القنوية
٢٧٨٠٤٠٥١٤٠٢٢٦٨
JS3328649






Name : SALWA AHMED MOUSALLA

Date : 05-06-2022

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS: -**

Chest:

- Status post left mastectomy showing small operative bed thick walled seroma measuring about 3.3 x 2 cm.
- Multiple sub pleural fine reticular bands are seen at anterior segment of left upper lobe.
- Few right middle lobe thick atelectatic bands.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

Abdomen:

- The liver is enlarged in size with homogenous low CT texture. No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Bulky uterus with IUD seen inside.
- Right adnexal small cyst is seen measuring about 4 cm.
- Reviewing skeletal parts revealed no lytic or sclerotic osseous lesions.

OPINION:

- **No CT evidence of pulmonary or hepatic metastatic disease.**

DR. Ibrahim Elsedawy.M.Sc, FEBR

Dr. Amr Farouk, MD

Name: سلوي احمد مصطفى		
MR. : 3183		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
femara calcium 1*1*90 one alpha zoladex 10.8 sc zometa 4 mg for ct chest paus a case of breast cancer under adj HT TAM SINCE 7/2019 THEN STHIFTED TO FEMARA ZOLADEX SINCE 6/2022 TILL NOW FOR FEMARA FOR 3 MONTH for femara / supp next 2 months at pharmacy for zoladex rt breast mamo dated 16/4/2026 shows normal study BIRAD 1 REPORT DONE FOR CT CAP APPROVAL 7/2026 C T CAP :FREE FEMARA +SUPPORTIVE*3M NEXT CLINIC :28/10/2026 Bone scan 27-8-2026 >>> supicious sacral , billateral SI uptake , pelvic MRI >> R to differntiate ossous lesion today for femara , zoladex , zometa , supp ttt after labs		
تحريرا في ,		
	الوقت :	التاريخ : 19-09-2026
	الطبيب المعالج : ايه مدحت بدوي محمد	
يعتمد ,		

خطابات أورام

NASSER INSTITUTE
ONCOLOGY CENTER

معهد ناصر
مركز الأورام



NUCLEAR MEDICINE UNIT

وحدة الطب النووي

Patient Name: Salwa Ahmed
Referring physician:

Date: 1/1/2020

^{99m}Tc -MDP BONE SCAN

Desired information: skeletal surveying to detect multiple lesions.

Technique: Anterior and posterior whole body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc -MDP;

Report:

- No definite hot spot that account for osseous deposits could be detected all over the surveyed skeleton.
- Degenerative changes are noticed over dorso-lumbar vertebrae & both knees.
- Both kidneys are seen

Conclusion:

- No definite evidence of osseous deposits.

Thanks for reference

Nuclear Medicine Unit

Magdy Koth, M.D.



Patient Name: Salwa Ahmed Moustafa Hussein Harb	Date : 20/06/2022
Exam : MAMMOGRAPHY BOTH	Age : 44 Year(s)
Medical ID: 2375/2018	Pat. ID: 13829

**DIGITAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY
ULTRASOUND**

Technique

- Right MLO and CC views
- Right 3D MLO DBT (Digital Breast tomosynthesis)
- MRM & breast ultrasound

Report:

- Left MRM:
 - Operative bed shows no mass or collection
 - No regional lymphadenopathy.
- Right Breast:
 - Predominant Fatty parenchyma (ACR A)
 - No suspicious micro calcifications or architectural distortion detected.
 - Complementary ultrasound no cystic or solid mass lesions.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Right axillary accessory breast.
 - Nonspecific axillary LN.

Opinion:

- Post left MRM status showing no evidence of recurrence (BIRADS 2)
- Normal sono-mammography of the right breast (BIRADS 1)

Signature Dr. Peter Nashat

20/06/2022 4:03 PM

Patient Name: Salwa Ahmed Moustafa Hussein	Date : 16/07/2025
Exam : ultrasound ABDOMEN& PELVIS	Age : 47 Year(s)
Medical ID: 2375/2018	Pat. ID: 13829

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- Enlarged in size (19cm at Rt MCL) bright parenchymal echopattern with smooth surface.
- No hepatic focal lesions or intra hepatic biliary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Bladder: -

- Not distended enough with small stone 5mm seen inside
- CBD is not dilated.

Spleen: -

Enlarged 14cm, homogenous.

Kidneys: -

- Both are average in size, shape and echogenicity
- No stones or dilated pelvicalyceal system
- No gross lesion

Para-aortic and pancreatic area: show no gross abnormalities.

No ascites

Urinary Bladder: -

- Average capacity with no stones or diverticular out pouching

Uterus and adnexal: no abnormalities could be detected apart from right ovarian simple cyst measuring 24x24mm

Opinion:

Bright hepatomegaly likely fatty

Mild splenomegaly

GB small stone

Right ovarian simple cyst likely functional for follow up

Signature Dr . Nermien Ramadan

16/07/2025 1:24 PM

Name: salwa ahmed mostafa
Date: 18 February 2020

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- enlarged in size (about 17 cm at the right MCL), smooth outline, bright echopattern.
- No intra hepatic biliary radicle dilatation.
- Portal vein is not dilated and patent .

Gall Bladder: -

- Average size, and wall thickness, no stone seen inside.
- CBD is not dilated.

Spleen: -

- Average size (About 10 cm), displaying homogenous echopattern.

Kidneys: -

- Are of normal size , site , well differentiated & with no gross stones or backpressure changes , no solid or cystic lesion.
- Para-aortic and pancreatic area: no gross lesion
- No ascites .

Urinary Bladder: -

- Average capacity with no stones, masses or diverticular out pouching

Uterus: average in size , endometrial line = 10 mm , no myometrial focal lesion.

Both adnexa:- no abnormalities could be detected

Opinion:

Bright enlarged liver.

Best regards
Dr: Doaa Ahmed Abdelaziz
MSc, FEBR



Patient Name: Salwa Ahmed Moustafa Hussein Harb	Date : 20/06/2022
Exam : MAMMOGRAPHY BOTH	Age : 44 Year(s)
Medical ID: 2375/2018	Pat. ID: 13829

**DIGITAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY
ULTRASOUND**

Technique

- Right MLO and CC views
- Right 3D MLO DBT (Digital Breast tomosynthesis)
- MRM & breast ultrasound

Report:

- Left MRM:
 - Operative bed shows no mass or collection
 - No regional lymphadenopathy.
- Right Breast:
 - Predominant Fatty parenchyma (ACR A)
 - No suspicious micro calcifications or architectural distortion detected.
 - Complementary ultrasound no cystic or solid mass lesions.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Right axillary accessory breast.
 - Nonspecific axillary LN.

Opinion:

- Post left MRM status showing no evidence of recurrence (BIRADS 2)
- Normal sono-mammography of the right breast (BIRADS 1)

Signature Dr. Peter Nashat

20/06/2022 4:03 PM



مستشفى أورام دار السلام

Dar El-Salam hospital

Radiology Department

قسم الأشعة

Name; seham farag mahmoud

Date : 04/07/22

Trans-vaginal ultrasound scanning revealed: -

○ **Uterus: -**

- *AVF, bulky uterus for age measuring about: 10x6.5x5cm in its longitudinal, transverse & AP diameters respectively.*
- *The myometrium shows homogenous texture with no focal lesion*
- *The endometrial line is 11mm*

○ **Ovaries: -**

- *Both ovaries are average size with no focal lesion*

- *No gross adnexal soft tissue masses or cysts.*
- *No free fluid in the Douglas pouch.*

Opinion

Bulky uterus

thickened endometrium

Best regards

D^r / NERMEEN RAMADAN.MD



Patient Name: Salwa Ahmed Moustafa Hussein Harb	Date : 04/06/2024
Exam : MAMMOGRAPHY BOTH	Age : 46 Year(s)
Medical ID: 2375/2018	Pat. ID: 13829

**DIGITAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY
ULTRASOUND**

Indication:

Patient is 65years old and is seen for follow-up after left MRM

Report:

- Left MRM:
 - Operative bed shows no mass or collection
 - No regional lymphadenopathy.
- Right Breast:
 - Predominant Fatty parenchyma (ACR A)
 - No suspicious micro calcifications or architectural distortion detected.
 - Complementary ultrasound no cystic or solid mass lesions.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Nonspecific axillary LN.

Opinion:

- Post left MRM status showing no evidence of recurrence (BIRADS 2)
- Normal sonomammography of right breast (BIRADS 1)

Best regards,
Dr Eman Abd ELrazek MSc, FEBR
Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCI, Cairo University
04/06/2024 3:53 PM

Name: salwa ahmed mostafa
Date: 18 February 2020

Soft tissue ultrasound examination of both breasts revealed: -

Follow up after left total mastectomy:

Left breast:

- > Skin thickening seen at site of scar, with localized fluid collection seen at the under outer margin of the scar and extended to the left axilla, its volume = 4.5 ml.
- > No definite solid mass.
- > No axillary LN.

right breast:

- > Predominantly fibro-glandular breast parenchyma with no areas of architectural distortion.
- > No masses or cysts could be detected.
- > Prominent duct system.
- > Normal nipple areola complexes.
- > Intact skin and subcutaneous tissues.

OPINION:

Localized collection under the operative bed (seroma) for follow-up.

Best regards

Dr / Doaa Ahmed MS-FEBR

The Breast Imaging and Reporting Data System for classifying the radiological diagnosis of breast lesions:

- BIRADS 0: needs further investigations (0%).
- BIRADS 1: normal findings.
- BIRADS 2: definitely benign.
- BIRADS 3: probably benign (2 %).
- BIRADS 4: probably malignant (50 %).
- BIRADS 5: malignant (98-100 %).
- BIRADS 6: proven pathologically malignant.

Pathology Report

Name: سلوى احمد مصطفى

Age: 40 years

Clinical data: Left breast mass.

Specimen Type: Tru-cut biopsy.

Date: 16/ 10 /2018

Lab. Number: 600

File Number: 2375/2018

Gross Description:

Three gray white tissue cores, 1cm each. Totally submitted.

Microscopic Examination:

Section examined revealed a case of invasive duct carcinoma with desmoplastic stroma. The tumour cells arranged in cords and trabeculae with moderate anaplasia. Dense sclerosis also could be detected.

Diagnosis:

Left breast mass, tru-cut biopsy.

Invasive duct carcinoma grade II with dense sclerosis.

ER,PR and Her2 are available on request.

Prof. Dr. Amr El Sabea

د. عمرو السباعي

Dr. Inas Mohamed Ali

Pathology Report

Name: سلوى احمد

Age: 40 years

Clinical data: Left breast cancer.

Specimen Type: Left modified radical mastectomy & left apical lymph node.

Date: 30/ 1 /2019

Lab. Number: 48

Hospital Number: 2375/2018

Gross Description:

Two specimens were received:

1- Modified radical mastectomy, breast tissue; 26x23x5.5 cm. skin ellipse; 22.5x15 cm, including nipple and areola. The nipple showed central roughness. The breast showed an ill-defined grayish white indurated areas 4.8x2.5 cm, located 2.5 cm, deep to skin, 1.5 cm away from the nipple with least deep margin 1.3 cm. separately received axillary tissue 11x9 cm, entangling 16 nodules largest 2.5x1.5 cm.

2- Left apical lymph node: Fibrofatty pieces, collectively 3x3 cm entangling one nodule 1x1 cm, totally submitted.

Microscopic Examination:

Histopathological examination of sections prepared from the tumour mass revealed an infiltrating malignant epithelial growth displayed dominantly as compressed angulated tubules and small clusters of pleomorphic hyperchromatic malignant ductal epithelial cells. Stromal desmoplasia is moderate. Cellular and nuclear atypia are moderate GII.

Minimal intraductal component, solid & cribriform types could be detected representing <5% of total surface area.

Lymphovascular emboli were seen.

Occasional perineural invasion was observed.

Axillary lymph nodes showed metastatic deposits in 6/16 nodes with focal extracapsular extension. Apical node was free (0/1) with surrounding free fibrofatty tissue. Total nodal status (6/17). The nipple, areola and surgical margins were free.

gnosis:

Modified radical mastectomy & left apical lymph node, invasive duct carcinoma GII, with wide tubular features.

Minimal intraductal component (non-comedo), <5% of tumour area.

Tumour necrosis index <15% of the tumour area.

Positive lymphovascular emboli, occasional perineural invasion.

* Free nipple, areola and soft tissue margins.

Axillary lymph nodes showed metastatic deposits in (6/16) nodes with focal capsular extension.

Apical node was free (0/1) with surrounding free fibrofatty tissue.

Total nodal status (6/17)

Stage T2N2MX.

Comment:

Minimal effect of previous treatment (Stromal Fibrosis) could be detected

Prof. Dr. Lobna Omar

د. لبنى عمر

ied radical mastectomy & left apical lymph node, invasive duct
GII, with wide tubular features.

intraductal component (non-comedo), <5% of tumour area.

necrosis index <15% of the tumour area.

lymphovascular emboli, occasional perineural invasion.

ple, areola and soft tissue margins.

/ lymph nodes showed metastatic deposits in (6/16) nodes with focal
ular extension.

node was free (0/1) with surrounding free fibrofatty tissue.

odal status (6/17)

12N2MX.

nt:

effect of previous treatment (Stromal Fibrosis) could be detected

Prof. Dr. Lobna Omar

د. لبنى عمر

Pathology Report

ie: سلوى احمد

Age: 40 years

ical data: Left breast cancer.

ecimen Type: Left modified radical mastectomy & left apical lymph node.

ate: 30/ 1 /2019

ab. Number: 48

Hospital Number: 2375/2018

Gross Description:

Two specimens were received:

- Modified radical mastectomy, breast tissue; 26x23x5.5 cm. skin ellipse; 12.5x15 cm, including nipple and areola. The nipple showed central roughness. The breast showed an ill-defined grayish white indurated areas 4.8x2.5 cm, located 2.5 cm, deep to skin, 1.5 cm away from the nipple with least deep margin 1.3 cm. separately received axillary tissue 11x9 cm, entangling 16 nodules largest 2.5x1.5 cm.
- 2- Left apical lymph node: Fibrofatty pieces, collectively 3x3 cm entangling one nodule 1x1 cm, totally submitted.

Microscopic Examination:

Histopathological examination of sections prepared from the tumour mass revealed an infiltrating malignant epithelial growth displayed dominantly as compressed angulated tubules and small clusters of pleomorphic hyperchromatic malignant ductal epithelial cells. Stromal desmoplasia is moderate. Cellular and nuclear atypia are moderate GII.

Minimal intraductal component, solid & cribriform types could be detected representing <5% of total surface area.

Lymphovascular emboli were seen.

Occasional perineural invasion was observed.

Axillary lymph nodes showed metastatic deposits in 6/16 nodes with focal extranodular extension. Apical node was free (0/1) with surrounding free fibrofatty tissue. Total nodal status (6/17). The nipple, areola and surgical

Diagnosis:

Left modified radical mastectomy & left apical lymph node, invasive duct carcinoma GII, with wide tubular features.

- * Minimal intraductal component (non-comedo), <5% of tumour area.
- * Tumour necrosis index <15% of the tumour area.
- * Positive lymphovascular emboli, occasional perineural invasion.
- * Free nipple, areola and soft tissue margins.
- Axillary lymph nodes showed metastatic deposits in (6/16) nodes with focal extracapsular extension.
- * Apical node was free (0/1) with surrounding free fibrofatty tissue.
- * Total nodal status (6/17)
- * Stage T₂N₂MX.

*please comment on
Clinical response ?!*

Prof. Dr. Lobna Omar

د. لبنى عمر

*Comm. minimal effect of prev treat-
ment (stromal) of previous treatment
(stromal fibrosis) could be
detected*

0114254437
01064585630
محمد سليم

مركز أورام دار السلام
إدارة المعمل
قسم الباثولوجي

Pathology Report

سلوى احمد

Age: 40 years

History: Left breast cancer.

Operation Type: Left modified radical mastectomy & left apical lymph node.

30/1/2019

Number: 48

Hospital Number: 2375/2018

Specimen Description:

Specimens were received:

Left modified radical mastectomy, breast tissue; 26x23x5.5 cm. skin ellipse; 15x15 cm, including nipple and areola. The nipple showed central roughness. The breast showed an ill-defined grayish white indurated areas 4.8x2.5 cm, 2.5 cm, deep to skin, 1.5 cm away from the nipple with least deep margin 1.3 cm. separately received axillary tissue 11x9 cm, entangling 16 nodules largest 2.5x1.5 cm.

Left apical lymph node: Fibrofatty pieces, collectively 3x3 cm entangling one nodule 1x1 cm, totally submitted.

Microscopic Examination:

Histopathological examination of sections prepared from the tumour mass revealed an infiltrating malignant epithelial growth displayed dominantly as compressed angulated tubules and small clusters of pleomorphic hyperchromatic malignant ductal epithelial cells. Stromal desmoplasia is moderate. Cellular and nuclear atypia are moderate. Minimal intraductal component, solid & cribriform types could be detected representing <5% of total surface area. Lymphovascular emboli were seen. Occasional perineural invasion was observed. Axillary lymph nodes showed metastatic deposits in 6/16 nodes with focal extracapsular extension. Apical node was free (0/1) with surrounding free fibrofatty tissue. Total nodal status (6/17). The nipple, areola and surgical margins were free.